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Who this book is for. Licensed or appropriately supervised behavioral health professionals (social workers, counselors, marriage and family therapists, psychologists, psychiatric nurse practitioners, and the programs that employ them), working within their scope of practice and their program's policies.

This book is not a clinically validated program, and it is not endorsed, certified, or approved by any institution, licensing board, or treatment-model owner. It does not replace individualized clinical judgment, assessment, diagnosis, treatment planning, supervision, or your program's emergency procedures. It is not certified training in DBT, CBT, ACT, Behavioral Activation, or any other named protocol; skills drawn from these approaches are adapted for general group facilitation.

No outcome for any client or program is promised. Sections on safety, crisis response, medication, trauma, and telehealth are marked [clinical] and cited to the sources in Appendix E.

In an emergency, follow your program's procedures and call local emergency services. In the United States, call or text 988 to reach the Suicide & Crisis Lifeline. Readers elsewhere should record their local crisis resources before using this book (see the Localization Checklist in Chapter 5).

First edition, 2026

Revision notice: This text was edited from the 272-page PDF supplied by the author. Dr. Salwa Ismael Hegazy reviewed and approved the revised manuscript, as confirmed by the author. Other reviewer credits refer to the original manuscript. Manuscript review is distinct from empirical validation of a treatment protocol.

Appendix D: Eight Reproducible Forms

These are printable masters, not descriptions of layouts. Form 1 has five pages and Form 4 has two. Complete program-specific fields and approve local privacy, consent, record, emergency, and clinical workflows before use. Blank lines are intentional response fields. The matching standalone pack contains the same masters. Forms do not establish medical necessity, validate a program, or replace required individual assessment and records.

PRINTABLE RESOURCES

Download clean copies of the eight reproducible forms and the program implementation forms.
Follow the book's permissions and printing instructions.

thoughtkindpress.com/resources/open-enrollment/volume-1/

Form 1: Rotation Passport - Process

Client-held | Passport page 1 of 5 | Attendance is not a test of mastery.

Program-approved identifier (may still be identifiable): _____ Admission date: _____ IOP / PHP

_____

Code	Topic	Date attended / revisit	Staff initials
PR-01	Arriving	_____	_____
PR-02	Carrying the Week	_____	_____
PR-03	The Mask and the Mirror	_____	_____
PR-04	Frustration and Stuckness	_____	_____
PR-05	Trust and Safety Here	_____	_____
PR-06	The Inner Critic	_____	_____
PR-07	Grief, Loss, and What Changed	_____	_____
PR-08	Hope and Doubt	_____	_____
PR-09	Connection and Isolation	_____	_____
PR-10	Resilience and Quiet Wins	_____	_____
PR-11	Giving and Receiving Feedback	_____	_____
PR-12	Anger and Boundaries	_____	_____
PR-13	Shame and Self-Forgiveness	_____	_____
PR-14	Identity Beyond the Diagnosis	_____	_____
PR-15	Navigating Outside Relationships	_____	_____
PR-16	Setbacks and Recovery Momentum	_____	_____
PR-17	Preparing for Transition / Stepping Down	_____	_____
PR-18	Endings, Goodbyes, and Continuity	_____	_____

An attendance date records attendance only. Note a revisit as such; do not infer understanding, clinical readiness, or completion of a full treatment model. Keep this document private. You may request staff help with recording or reading it.

Form 1: Rotation Passport - Skills

Client-held | Passport page 2 of 5 | Attendance is not a test of mastery.

Program-approved identifier (may still be identifiable): _____ Admission date: _____ IOP / PHP

_____

Code	Topic	Date attended / revisit	Staff initials
SK-MF-01	Observe and Describe	_____	_____
SK-MF-02	Grounding When Overwhelmed	_____	_____
SK-MF-03	Attention Training	_____	_____
SK-ER-01	Emotions, Urges, and Actions	_____	_____
SK-ER-02	Check the Facts and Opposite Action	_____	_____
SK-ER-03	Reducing Emotional Vulnerability	_____	_____
SK-DT-01	Crisis Survival Skills	_____	_____
SK-DT-02	Radical Acceptance and Turning the Mind	_____	_____
SK-DT-03	Self-Soothing with the Senses and a Distraction Plan	_____	_____
SK-CG-01	The Thought-Feeling-Behavior Connection and Common Thinking Traps	_____	_____
SK-CG-02	Examining the Evidence and Thought Reframing	_____	_____
SK-CG-03	Cognitive Defusion and Noticing Hooked Thoughts	_____	_____
SK-BA-01	Activity Scheduling and the Pleasure-Mastery Balance	_____	_____
SK-BA-02	Values-Based Action and Overcoming Avoidance	_____	_____
SK-BA-03	Graded Task Assignment and Habit Stacking	_____	_____
SK-IE-01	Objective Effectiveness and Assertive Communication	_____	_____
SK-IE-02	Relationship Effectiveness and Boundary Setting	_____	_____
SK-IE-03	Self-Respect Effectiveness and Saying No	_____	_____

An attendance date records attendance only. Note a revisit as such; do not infer understanding, clinical readiness, or completion of a full treatment model. Keep this document private. You may request staff help with recording or reading it.

Form 1: Rotation Passport - Psychoeducation

Client-held | Passport page 3 of 5 | Attendance is not a test of mastery.

Program-approved identifier (may still be identifiable): _____ Admission date: _____ IOP / PHP

_____

Code	Topic	Date attended / revisit	Staff initials
PE-01	Understanding Depression	_____	_____
PE-02	Understanding Anxiety and the Fight-or-Flight Response	_____	_____
PE-03	Chronic Stress and Recovery Time	_____	_____
PE-04	The Panic Cycle and Interoceptive Conditioning	_____	_____
PE-05	Sleep and Mood	_____	_____
PE-06	Strong Emotions and Making Room to Pause	_____	_____
PE-07	The Window of Tolerance and Autonomic Regulation	_____	_____
PE-08	Understanding Trauma, Triggers, and the Nervous System	_____	_____
PE-09	Shame, Self-Criticism, and Self-Compassion	_____	_____
PE-10	Values, Meaning, and Personal Direction	_____	_____
PE-11	Understanding Anger and Emotional Triggers as Functional Signals	_____	_____
PE-12	Grief, Loss, and Non-Death Transitions	_____	_____
PE-13	Loneliness and Meaningful Connection	_____	_____
PE-14	Interpersonal Boundaries and Relationship Systems	_____	_____
PE-15	Executive Dysfunction, Task Paralysis, and Motivation	_____	_____
PE-16	Building Resilient Support Networks and Crisis Navigation	_____	_____
PE-17	Relapse Prevention: Lapses, Relapses, and Recovery Curves	_____	_____
PE-18	Stepping Down: Maintenance, Care Continuity, and Generalization	_____	_____

An attendance date records attendance only. Note a revisit as such; do not infer understanding, clinical readiness, or completion of a full treatment model. Keep this document private. You may request staff help with recording or reading it.

Form 1: Rotation Passport - PHP Extension

Client-held | Passport page 4 of 5 | Attendance is not a test of mastery.

Program-approved identifier (may still be identifiable): _____ Admission date: _____ IOP / PHP

_____

Code	Topic	Date attended / revisit	Staff initials
PH-WS-01	Daily Rhythm Anchors	_____	_____
PH-WS-02	Eating Routines, Access, and Daily Energy	_____	_____
PH-WS-03	Building an Evening Wind-Down	_____	_____
PH-WS-04	Daily Demands and Energy Pacing	_____	_____
PH-WS-05	Medication Adherence and Prescriber Partnership	_____	_____
PH-WS-06	Rest, Withdrawal, and What I Need	_____	_____
PH-SP-01	Deconstructing the Crisis Trajectory: Micro-Triggers and Prodromal Signs	_____	_____
PH-SP-02	Internal Coping vs. Social Distraction	_____	_____
PH-SP-03	Environmental Safety and Means Restriction	_____	_____
PH-SP-04	The Anatomy of an Urge: Urge Surfing and Cognitive Decoupling	_____	_____
PH-SP-05	Post-Lapse Cognitive Processing: Reducing the Abstinence Violation Effect	_____	_____
PH-SP-06	Reviewing Safety-Plan Access, Contacts, and Gaps	_____	_____
PH-TS-01	Questions About Levels of Care and Stepping Down	_____	_____
PH-TS-02	Roles and Identity Beyond Treatment	_____	_____
PH-TS-03	My Routine and Check-In Plan	_____	_____
PH-TS-04	Outpatient Follow-Up and Handoff Questions	_____	_____
PH-TS-05	Mixed Feelings About Leaving	_____	_____
PH-TS-06	My Recovery Reference	_____	_____
PH-AP-01	Practicing a Request or Boundary	_____	_____
PH-AP-02	Noticing Mild Body Sensations and Grounding	_____	_____
PH-AP-03	Working With a Manageable Current Thought	_____	_____
PH-AP-04	Taking a First Step in the Room	_____	_____
PH-AP-05	Safety-Plan Access Practice	_____	_____
PH-AP-06	Adapting a Skill for Daily Life	_____	_____

An attendance date records attendance only. Note a revisit as such; do not infer understanding, clinical readiness, or completion of a full treatment model. Keep this document private. You may request staff help with recording or reading it.

Form 1: Passport - Special Plans and Personal Notes

Client-held | Passport page 5 of 5

Program-approved identifier:

_____

Code	Topic	Date / not applicable	Staff initials
SP-OR	Day-One Program Orientation and Expectation Setting	_____	_____
SP-SD	A Private Transition Review and Optional Group Goodbye	_____	_____
SP-CD	Support After a Difficult Group Event	_____	_____
SP-AV	Low Attendance and Program Disruption	_____	_____

PHP track coverage - count distinct codes actually attended, with staff assistance if wanted:

Track	Distinct codes attended (0-6)	Questions / personal notes
Wellness and daily structure	_____	
Safety and relapse planning	_____	
Transition and step-down	_____	
Applied practice	_____	

Notes to myself:

_____

_____

_____

_____

_____

_____

_____

Bring your Passport when it is useful. At transition, review gaps privately with staff. Take-home information does not replace a session, individualized care, or a reviewed clinical safety plan.

Form 2: Attendance and Rotation Slot Tracker

Staff-held | One program day | Store in an approved restricted location.

Program / date / core day index / extension day index / staff:

_____

_____

Slot	Planned code	Actual code / format	Actual start-stop
G1			
G2			
G3			
G4			
G5			

Client ID	G1	G2	G3	G4	G5	New / transition
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____

Use program-approved attendance codes and distinguish absent, present, partial, declined, and not offered. Actual service times and individual notes are maintained in the required clinical records.

Closure / substitution / reason and next operational slot:

_____

_____

_____

Completed by / reviewed by / date:

_____

Form 3: Group Agreement and Confidentiality Discussion

Review with staff at orientation | Offer a copy | Supplement with the program's required informed consent.

Client identifier / staff name and role / date:

_____

Please ask questions or request an accessible explanation. This page acknowledges a discussion; it does not waive care, privacy rights, or the right to raise concerns.

Privacy and its limits

[ ] I am asked not to repeat identifiable information about others, record, photograph, or share screenshots. Staff cannot guarantee that other participants will keep information private. I can ask what participation options fit my privacy needs.

[ ] Staff explained relevant treatment-team sharing, record access, and applicable reporting, emergency, or protective exceptions. The program's jurisdiction-specific consent process governs those details.

Program-specific limits explained / questions answered / language or interpreter support:

_____

_____

_____

Participation, respect, and access

[ ] I can choose what to disclose and may request listening, writing, demonstration, an adapted activity, or a supported pause. Attendance and treatment expectations, and any limits on these choices, have been explained separately. I will not threaten, harass, or pressure another person to disclose.

[ ] We ask permission before advice or feedback. Peers do not provide risk assessment, medication advice, or crisis care. Staff retain responsibility for clinical decisions.

[ ] Staff explained how to reach help during group, between sessions, and in an emergency. My current contact card is available in an accessible form.

Local norms, access needs, camera/chat/recording arrangements if relevant:

_____

_____

_____

Teach-back: how I can request help or a private concern:

_____

_____

Client acknowledgment / date / staff acknowledgment / date (or document verbal review per policy):

_____

_____

Form 4: Session Delivery and Secure Handoff

Staff-held | Group operations and pending clinical actions | Keep personal details in approved clinical channels.

Date / facilitator / co-facilitator / planned and actual code / actual format and time:

_____

_____

What was actually delivered; adaptations; attendance count; displaced code or closure:

_____

_____

_____

Group dynamic or access issue relevant to the next facilitator (avoid unnecessary identifiers):

_____

_____

_____

Closed-loop clinical handoff - use minimum necessary information under applicable policy

Client ID / secure record pointer	Concern / action already taken	Receiving clinician / due time	Acknowledged / contingency

Immediate unresolved issue, next action, responsible person, acknowledgment time:

_____

_____

_____

An urgent concern requires direct clinical communication now, not a form left for the next shift. Record clinical assessment and actions in the required individual record; do not place another member's information in a patient's note.

Outgoing staff / time; incoming staff / time; acknowledgment or alternate recipient:

_____

_____

Form 4: Individual Encounter Note - GIRP or DAP

One person and one actual encounter | Alternative formats; choose the format required locally.

Client ID / date / actual service, code, location, and start-stop time / clinician:

_____

_____

GIRP fields

G - Individual treatment goal and relevance of this encounter:

_____

_____

I - Intervention actually delivered, dose/time, accommodations, and format:

_____

_____

_____

R - Observed behavior / performance / declined or not assessed; then separately attributed member report:

_____

_____

_____

_____

P - Agreed next step or no step agreed; assistance, referral, owner, due time, acknowledgment:

_____

_____

_____

DAP alternative

D - Actual intervention, attendance/time, observed behavior, and attributed report:

_____

_____

_____

A - Interpretation linked to an individual goal, with the supporting observation; state limits:

_____

_____

P - Individual next step and any clinical action, owner, due time, handoff:

_____

_____

Do not infer improvement, safety, engagement, understanding, or readiness from attendance or a worksheet. Record urgent actions as they occur. This is not a completed note, validated assessment, or billing assurance.

Form 5: Coverage Review and Continuity Assignment

Staff and member, privately | At transition or a planned review | Coverage is separate from mastery.

Client ID / date / reviewed with / receiving provider if applicable:

_____

_____

Codes actually offered during stay: ____ Distinct codes attended: ____ Revisits: ____

_____

Code / gap	Summary or practice material offered	Date / accessible format	Clinical review or follow-up owner

Skills summaries in the companion workbook may support a discussion, but do not replace missed treatment. Select only material that fits the member's needs and the receiving clinician's plan. Do not issue a self-guided exposure task or an incomplete safety plan as a catch-up assignment.

Reason for gap, if useful for program review (not a failure label):

_____

_____

Appointment accepted / date / access barriers / responsible staff / due time:

_____

_____

_____

Receiving-service contact and handoff acknowledgment, under the applicable consent process:

_____

_____

Form 6: Crisis and Support Contact Card

Client-held | Complete local fields with staff, verify, and cut or copy the card | A contact card is not a safety plan.

Staff check before issue: verify location served, hours, eligibility, emergency route, accessible format, and backup. Do not hand out blank program contact fields as if they were verified resources.

CRISIS AND SUPPORT CARD - FRONT	MY SUPPORT CONTACTS - BACK
United States: call or text 988 for the Suicide & Crisis Lifeline. For immediate danger or a medical emergency, contact local emergency services (911 in the U.S.) or follow the emergency route confirmed by your care team. Program: _____ Daytime: _____ After-hours / hours served: _____ Local emergency route: _____	My clinician: _____ Phone / hours: _____ Prescriber: _____ Phone / hours: _____ Support person, if agreed: _____ Phone / availability: _____ Backup service / hours: _____ Current reviewed plan location: _____ Date checked with staff: _____

Outside the United States, staff must substitute current resources for the participant's actual location. A phone being unavailable, a contact not answering, or a coping step not helping is a reason to use the backup or urgent help route, not to wait through a required sequence.

Verified by / date / participant location served / accessible version and backup:

_____

_____

_____

Form 7: Safety Escalation and Follow-Up

Staff-held | Supplement the required risk/incident record | Immediate communication when urgent.

Client ID / date / session code / facilitator / actual time concern arose:

_____

_____

Observed facts and separately attributed statement (no unsupported interpretation):

_____

_____

_____

_____

Immediate action / times / staff assigned / medical or emergency response if used:

_____

_____

_____

_____

Individual assessment completed by / secure record location / current disposition:

_____

_____

_____

Receiving clinician or supervisor / notification method and time / acknowledgment:

_____

_____

Unresolved action / owner / due time / contingency if the recipient cannot be reached:

_____

_____

_____

Participant follow-up and next contact; group support needs without other-member details:

_____

_____

_____

Facilitator signature/time / supervisor review/time:

_____

_____

Do not treat a checkbox, a promise, a grounding response, or a returned worksheet as evidence of low risk. The responsible clinician and program procedures determine assessment, monitoring, escalation, and follow-up.

Form 8: Your Experience of the Program

Optional | No name requested | Participation does not affect care.

Do not include another person's identifying information. Staff should explain how responses are collected and any limits on anonymity, especially in a small program. This survey is an editorial feedback aid, not a validated outcome measure.

Please circle one number, N/A, or skip any item. 1 = not helpful; 5 = very helpful. These are experience questions, not symptom scores.

Experience	1	2	3	4	5	N/A
Understanding how to join the active rotation						
Practical usefulness of the groups						
Being able to choose how to participate						
Facilitator response to my needs						
Usefulness of any workbook or handouts						
Preparation for the next stage of care						

What was useful for you?

_____

_____

_____

_____

What would you change or want handled differently?

_____

_____

_____

_____

Anything the program did not address, or an access need it missed?

_____

_____

_____

Optional context: IOP / PHP / prefer not to say. Please omit identifying details. For an urgent concern, contact program staff or the verified help route directly; this form is not monitored for emergency response.

Appendix G - Program Implementation Pack

Staff reference and blank local forms. Program decisions and approvals must be completed locally. These additions do not convert the curriculum into a validated treatment protocol.

Program setup: decisions before the first group

This pack turns the implementation questions into writable decisions. It is not a completed local SOP. Clinical leadership approves the local routes, permitted roles, staffing and record procedures before use. Keep it beside the session plan, not in a client workbook.

Program / site / jurisdiction / version / review date

_____

_____

_____

Clinical approver and authorized roles; relevant license or supervision requirements

_____

_____

_____

Group purpose, maximum size and minimum staffing; reason for these choices

_____

_____

_____

Named backup, contact method, hours and alternative if unavailable

_____

_____

_____

Before opening a group, verify coverage for both the person needing support and the remaining group. If the planned backup is absent, obtain an approved alternative or change/postpone the session under site policy. Do not improvise a routine group without the coverage it requires. An emergency that occurs anyway calls for the emergency route immediately.

Local safety addendum 1: urgent routes

Complete each route with a named role, working contact, immediate action, coverage of the group, and handoff destination. Test contact details privately. Do not put participant identities on this blank staff reference.

Medical emergency / suspected ingestion / breathing or consciousness concern

_____

_____

_____

Suicide or self-harm concern: assessor / emergency route / staff coverage

_____

_____

_____

Threats or violence: protection / security or emergency route / protected record

_____

_____

_____

Intoxication, withdrawal or possible medication reaction: medical route

_____

_____

_____

Acute psychosis, mania, severe disorganization or eating-related medical concern: qualified assessment route

_____

_____

_____

A routine worksheet is not the response to an acute medical or psychiatric concern. Activate the appropriate local response promptly; do not wait an arbitrary number of minutes for a callback. The program determines monitoring, transfer and return arrangements.

Local safety addendum 2: continuity and privacy

Remote: verified current location, safe callback and local responder; disconnection plan

_____

_____

_____

Safeguarding / mandated reporting / duty-to-protect consultation route

_____

_____

_____

Private space and staff coverage; leaving / rejoining arrangements

_____

_____

_____

Records owner, approved system, access, retention and secure disposal policy

_____

_____

_____

Incident notification, review owner and follow-up deadline

_____

_____

_____

The sending staff member states the concern, actions taken, uncertainties, next action and contingency. The receiving staff member confirms understanding and accepts the specified responsibility; record who accepted what and when. Sending a message alone does not complete a handoff. See AHRQ, Tool: Handoff.

Staff explain applicable confidentiality limits and their reporting duties before participation. Ask members to protect privacy, while explaining that staff cannot guarantee other members will do so. Resolve profession- and jurisdiction-specific questions through the program.

Fit for this group: a private clinical decision

Assess the person and the actual group task together. A diagnostic label, neurodivergence, a communication difference or limited literacy is not an automatic exclusion. AGPA discusses the interaction of client fit, group composition and leader competence.

Person / approved identifier / date / responsible clinician

_____

_____

Relevant care goal, preferred participation and access or interpretation needs

_____

_____

_____

Current capacity for this task; acute concerns needing individual assessment first

_____

_____

_____

Decision: proceed / adapt / defer to another service; reason and alternative support

_____

_____

_____

Staffing and space needed; person responsible; review / return criteria

_____

_____

_____

Acute withdrawal, possible medical instability, severe mood elevation, psychosis-related distress or severe disorganization require assessment of current needs. Do not equate a calm appearance, a rating, attendance or worksheet completion with suitability or permission to return.

One-page code implementation plan

Copy this form for each code in use. Consult the code matrix for its specific competence focus. Approval belongs to the adopting program; this blank form does not certify anyone.

Code / title / purpose linked to care plans / approved version

_____

_____

Permitted lead roles and demonstrated task competencies

_____

_____

Co-facilitator or direct supervisor; when they must be present

_____

_____

Local SOP / stop signs / immediate action / qualified recipient and contact

_____

_____

_____

Group coverage / private support / accepted handoff and return criteria

_____

_____

_____

One demonstration / complete practice / debrief / close; access alternatives

_____

_____

_____

Clinical approver / signature or approved-system confirmation / date / review date

_____

_____

Minimum viable session: keep the useful work

Choose one aim and one complete practice. A short group should still let a person try something, notice what happened and leave with a support route. Do not try to deliver a whole long plan faster.

Shorter skills, psychoeducation and PHP routes (minutes: opening / demonstration / one complete practice / debrief / close): 30 = 4 / 4 / 12 / 5 / 5; 45 = 6 / 9 / 18 / 7 / 5; 50 = 7 / 10 / 20 / 8 / 5; 60 = 8 / 12 / 25 / 10 / 5; 90 = 12 / 18 / 35 / 15 / 10. Choose one focused task; shorten explanation and extra examples before cutting the practice or close. Safety checks and access adaptations apply throughout. These are facilitation examples, not billing rules. Use the separate process-group route for PR work.

Today's code / care-related aim / time available

_____

_____

Fictional demonstration in ordinary words; materials already available

_____

_____

_____

One complete practice and an equivalent spoken, shown or observed option

_____

_____

_____

Debrief: what fit, what did not, what needs changing?

_____

_____

Close: optional next step, question or support; owner of unresolved concerns

_____

_____

_____

Example: for a clear request, show one short request, let the member change the words, try it once, then ask whether it fits their actual relationship and circumstances. A second example is optional; the first complete attempt matters more.

Staff handoff: confirm who owns the next action

Professional use only. Complete through the approved private system. A Passport or client sheet is not the clinical handoff.

Sender / receiver / time / approved identifier / code

_____

_____

Observed behavior and attributed client report; current concern and uncertainty

_____

_____

_____

Intervention and adaptation actually used; response actually observed or reported

_____

_____

_____

Relevant care-plan or safety-plan location; pending contact or referral

_____

_____

_____

Next action / named owner / timeframe / contingency if no response

_____

_____

_____

Receiver's confirmation of understanding and accepted responsibility / time

_____

_____

_____

Avoid copied claims of engagement, recovery or consent that were not established. Exclude unrelated peer histories and unnecessary identifiers. Include necessary, relevant safety information in the appropriate protected record as permitted or required; do not apply a blanket ban that erases essential facts.

QI record 1: delivery and access

This is a local improvement form, not a validated outcome scale. Keep program review data de-identified where possible and separate from the clinical record.

Review period / owner / groups sampled / sampling method

_____

_____

Attendance: attended opportunities / offered opportunities; define exclusions

_____

_____

Retention: your local definition, eligible denominator and count; reasons known or unknown

_____

_____

Delivery: Bridge, meaningful choice, one complete practice, appropriate safeguards, supported close; completed / adapted / not delivered / not applicable

_____

_____

_____

Access barrier, declined activity or adaptation; what the team will change

_____

_____

_____

Supervision discussion / action owner / date to check the change

_____

_____

_____

Report the denominator and missing information. Attendance is not improvement; a fidelity tick is not evidence of clinical benefit. Review the work with supervision rather than scoring staff from an isolated incident.

QI record 2: experience, concerns and goals

Period / reviewer / data source / privacy arrangements

_____

_____

Adverse event or near miss: category, immediate action, responsible reviewer; detailed protected incident record location

_____

_____

_____

Optional participant experience: what helped? what felt unhelpful or unsafe? preferred change or private route

_____

_____

_____

Individualized goal: measure chosen in the care plan, baseline, follow-up, missing data and interpretation by the responsible clinician

_____

_____

_____

Team learning: decision, owner, completion date, next review

_____

_____

_____

Offer anonymous or private feedback where feasible and explain its limits. Passing or missing feedback does not mean satisfaction. Do not infer safety, effectiveness or validated outcomes from this untested improvement pack. Do not pool identifiable information into shared training documents.

Localization and rehearsal record

Jurisdiction / local language / clinical and privacy approvers

_____

_____

Verified emergency and crisis contacts; service hours; safe remote-response route

_____

_____

_____

Applicable scope, reporting, consent, confidentiality and record requirements; policies checked and review date

_____

_____

_____

Translation / interpreter / cultural and sensory adaptations checked with intended users

_____

_____

_____

Rehearsal: lead, backup and group coverage; contact test; interrupted handoff; no-backup contingency

_____

_____

_____

Issues found, changes made, supervisor authorization and next review

_____

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Practice with fictional information. Rehearsal is preparation, not proof of competence or clinical effectiveness. A small supervised local introduction can identify usability problems; record what actually happened rather than claiming a pilot has already succeeded.